NURS 3120 Exam 3 Completed Study Worksheet

Health Assessment and Promotion

Review the answers by chapter. Each source line identifies the reference document and its PDF page number, including instructor notes where applicable. PDF pages are counted from the first page of the file, rather than the printed slide number.

Chapter 14 Skin

Define moisture associated skin damage and list risk factors

MASD is skin breakdown associated with moisture. Common sites are skin folds, beneath the breasts, under the pannus, and the buttocks. Moisture exposure and moisture trapped in folds increase risk; the chapter also emphasizes incontinence skin cleansing as a protective measure.

Source: Chapter 14 8th edition.pdf, PDF page(s) 29, 32.

List skin cancer risk reduction strategies

Reduce sun exposure and seek shade. Use sunscreen with SPF 15 or higher as specified in the course slides. Wear long sleeves, a wide brimmed hat, and sunglasses. Avoid sunburn and tanning booth exposure. Examine skin for changing lesions using ABCDE. Maintain adequate niacin intake.

Source: Chapter 14 8th edition.pdf, PDF page(s) 13, 23.

Differentiate pressure injury stages

Stage 1: Intact skin with redness that does not blanch with pressure.

Stage 2: Partial thickness skin erosion; the first layer of skin is broken. An intact blister may be present. Not every blister is a pressure injury.

Stage 3: Full thickness ulcer extending into subcutaneous tissue; craterlike appearance, visible fat, and possible tunneling.

Stage 4: Bone, tendon, or muscle is visible.

Unstageable: The wound base cannot be seen, preventing determination of depth; the notes describe black, necrotic tissue obscuring the wound.

Source: Chapter 14 8th edition.pdf, PDF page(s) 33–37.

Differentiate primary skin lesions

Papule/plaque: Elevated, palpable, solid lesions; examples include elevated nevi, warts, and psoriasis.

Nodule/tumor: Elevated, solid, palpable masses extending deeper into the dermis than papules; examples include lipoma and keloids.

Macule/patch: Flat, nonpalpable color changes. Macules are less than 1 cm with circumscribed borders; patches are greater than 1 cm and may have irregular borders.

Vesicle/bulla: Circumscribed, elevated, palpable lesions containing serous fluid; examples include herpes and burns.

Cyst: Encapsulated fluid filled or semisolid mass in the dermis or subcutaneous tissue.

Pustule: Pus filled vesicle or bulla, such as acne.

Wheal: Elevated, often irregular mass with transient borders and no free fluid, such as hives or insect bites.

Source: Chapter 14 8th edition.pdf, PDF page(s) 14–15.

List factors that increase pressure injury risk

Unrelieved pressure, prolonged bed rest, limited activity or mobility, impaired sensory perception, moisture, inadequate nutrition, friction, and shear. The Braden Scale assesses sensory perception, moisture, activity, mobility, nutrition, and friction/shear; lower scores mean greater risk.

Source: Chapter 14 8th edition.pdf, PDF page(s) 30–32.

Chapter 20 Breast and Lymphatic System

Describe normal versus abnormal breast findings

Normal: Smooth skin without edema; breasts may differ somewhat in size. Symmetric movement without dimpling or retraction; smooth, firm, elastic tissue at normal body temperature. No masses or tenderness, although generalized tenderness may occur with menstruation or hormonal medication. Longstanding congenital nipple inversion can be normal. Axillary nodes may be nonpalpable or one to two small, discrete, nontender, movable central nodes.

Abnormal: New unilateral enlargement, dimpling, retraction, a newly inverted nipple, peau d'orange, nipple/areolar scaling or crusting, redness and pain, a hard irregular immobile mass, or hard fixed nodes. Spontaneous unilateral bloody or blood tinged nipple discharge requires evaluation.

Source: Chapter 20 8th edition.pdf, PDF page(s) 5–13, 23.

List breast cancer risk factors

Nonmodifiable: Female sex, age over 50, BRCA1/BRCA2 mutations, personal breast cancer history, menarche before 12, menopause after 52, and prior chest radiation before 40.

Modifiable or reproductive/lifestyle factors listed by the course: Nulliparity or first birth after 30, no breastfeeding history, postmenopausal hormone therapy, night shift work, alcohol intake of 2–5 drinks daily, excess weight/obesity, and inactivity.

Source: Chapter 20 8th edition.pdf, PDF page(s) 16–17.

Where are breast tumors most common

Upper outer quadrant, including the tail of Spence.

Source: Chapter 20 8th edition.pdf, PDF page(s) 15.

Define gynecomastia and describe findings and causes

Enlargement of male glandular breast tissue, felt as a smooth, firm, movable disc. It may occur temporarily during puberty and recur with aging as testosterone decreases relative to estrogen. Other associations include hormonal imbalance, drug abuse, cirrhosis, leukemia, thyrotoxicosis, and medications such as spironolactone,

amiodarone, calcium channel blockers, and hormones. A hard irregular nodule is a concerning alternative finding.

Source: Chapter 20 8th edition.pdf, PDF page(s) 14.

Describe breast examination technique

Inspect color, texture, size, symmetry, nipples, discharge, dimpling, and retraction. Inspect with arms overhead, hands pressed on hips, palms pressed together, and leaning forward with support for large breasts.

For palpation, position supine with the arm on the examined side overhead and a small pillow or rolled towel supporting that side. Use the pads of three fingers with light, medium, and firm pressure. Use a consistent vertical strip, concentric circle, or radial spoke pattern, covering the entire breast, areola, nipple, and tail of Spence. Assess tenderness, temperature, texture, and masses. Wear gloves for gentle nipple compression when assessing discharge. Assess axillary nodes with the client sitting and the arm supported.

Source: Chapter 20 8th edition.pdf, PDF page(s) 6–13.

Chapter 21 Cardiovascular

Where are S1 and S2 heard best and which valves are involved

S1 is loudest at the apex and marks the beginning of systole; it is produced by closure of the mitral and tricuspid valves. S2 is loudest at the base and marks the end of systole/beginning of diastole; it is produced by closure of the aortic and pulmonic valves.

The worksheet says “opening,” but the sounds correspond to closure. After S1, ventricular pressure opens the semilunar valves for ejection; after S2, the AV valves open as ventricular pressure falls and filling begins.

Source: Chapter 21 8th Edition.pdf, PDF page(s) 6, 24.

Identify the heart valve auscultation locations

Aortic: Second intercostal space, right sternal border.

Pulmonic: Second intercostal space, left sternal border.

Tricuspid: Fourth or fifth intercostal space, left lower sternal border.

Mitral: Fifth intercostal space near the left midclavicular line, at the apex.

Additional landmark: Erb point is the third intercostal space, left sternal border.

Source: Chapter 21 8th Edition.pdf, PDF page(s) 24.

Define jugular venous distention and signs of right sided heart failure

JVD is visible distention or bulging of the jugular veins from elevated venous pressure. It reflects increased pressure/volume on the right side of the heart and can indicate right ventricular failure. Other causes listed include pulmonary hypertension, pulmonary emboli, and cardiac tamponade. JVD with dependent bilateral leg/ankle edema supports concern for right sided congestion; the chapter describes cardiac edema as bilateral and worse later in the day.

Source: Chapter 21 8th Edition.pdf, PDF page(s) 14–16, 23.

Describe positioning for jugular venous pressure assessment

Place the client supine with the head of the bed elevated 30–45 degrees. Turn the head slightly away from the side being examined and use tangential lighting. The notes also describe observing for persistent distention as elevation increases from 30 to 45, 60, and 90 degrees.

Source: Chapter 21 8th Edition.pdf, PDF page(s) 16.

List four cardiovascular risk reduction strategies and nonmodifiable risks

1. Stop smoking.

2. Eat a healthy diet and reduce elevated cholesterol.

3. Increase physical activity and maintain a healthy weight.

4. Control hypertension and diabetes through prescribed care and lifestyle measures.

Nonmodifiable risks: Increasing age, sex assigned at birth, and heredity/family history. These cannot be reduced directly but help guide risk assessment.

Source: Chapter 21 8th Edition.pdf, PDF page(s) 31.

Describe carotid artery assessment technique

When cardiovascular disease is known or suspected, auscultate before palpation. Use the bell over each artery and ask the client to briefly hold their breath so breath sounds do not mask a bruit. No blowing or swishing should be heard. Then use the index and middle finger pads medial to the sternocleidomastoid muscle to palpate one artery at a time, assessing amplitude and any thrill. Never palpate both simultaneously because this can reduce cerebral blood flow.

Source: Chapter 21 8th Edition.pdf, PDF page(s) 18–20.

What palpation precaution is needed with known cardiovascular disease

Auscultate the carotids first. If auscultation suggests narrowing/occlusion, palpate very lightly to avoid blocking circulation or triggering vagal stimulation, bradycardia, and hypotension. Assess one side at a time.

Source: Chapter 21 8th Edition.pdf, PDF page(s) 18–20.

List chest pain assessment questions

Use COLDSPA: Character—What does the pain feel like? Onset—When and how did it begin? Location—Where is it, and does it radiate? Duration—How long does it last? Severity—How intense is it? Pattern—What triggers, worsens, or relieves it, including exertion, rest, breathing, position, and nitroglycerin? Associated factors—Any dyspnea, nausea/vomiting, sweating, weakness, or fatigue? Ask about cardiac history, medications, and risk factors.

Source: Chapter 21 8th Edition.pdf, PDF page(s) 11–13.

Compare types of chest pain

Angina: Tight, heavy, pressing discomfort, often with exertion; may radiate to shoulders, jaw, neck, or upper abdomen. Usually mild to moderate and lasts up to 20 minutes; may improve with nitroglycerin. Dyspnea, nausea, or sweating may occur.

MI: Heavy, tight, pressing discomfort with or without exertion; similar radiation, often moderate to severe, lasting 20 minutes to hours. Rest does not relieve it; nausea, vomiting, weakness, and sweating may occur.

Aortic dissection: Abrupt, severe tearing or ripping pain in the chest, radiating to neck, back, or abdomen; may persist for hours and requires treatment for relief.

Pericarditis: Sharp, stabbing or pounding pain, possibly radiating to shoulders/neck; worsens with breathing, coughing, or positional changes and may improve when sitting forward. Fever, weakness, fatigue, or dyspnea may accompany it.

Source: Chapter 21 8th Edition.pdf, PDF page(s) 13.

Chapter 22 Peripheral Vascular

Describe peripheral edema assessment and grading

Press over the pretibial area and assess for a persistent indentation after releasing pressure; note depth, rebound time, swelling, and whether it is unilateral or bilateral.

0: No pitting edema.

1+: Mild pitting, about 2 mm; disappears rapidly.

2+: Moderate pitting, about 4 mm; resolves in 10–15 seconds.

3+: Deep pitting, about 6 mm; persists about a minute or longer; visible swelling.

4+: Very deep pitting, about 8 mm; persists more than 2 minutes; gross swelling/distortion.

Source: Chapter 22 8th Edition.pdf, PDF page(s) 19; also Chapter 14 PDF p. 12 and Chapter 21 PDF p. 23.

What risk reduction education is needed for PAD

Stop smoking, control blood glucose, exercise regularly, maintain normal cholesterol, eat a healthy diet, and maintain a healthy weight. Teach the signs of worsening perfusion, including activity related leg pain, cool/color changed skin, and foot/toe wounds that do not heal; report changes.

Source: Chapter 22 8th Edition.pdf, PDF page(s) 7–11.

List PAD risk factors

Smoking, diabetes, coronary heart disease, obesity, hypertension, high cholesterol, family history, increasing age/vascular rigidity, African American ancestry as listed in the course, and elevated homocysteine.

Source: Chapter 22 8th Edition.pdf, PDF page(s) 9–10.

What findings suggest a ruptured abdominal aortic aneurysm

Sudden abdominal and back pain, clammy/sweaty skin, increased heart rate, and possible loss of consciousness in a client with an AAA. Rupture is life threatening.

Source: Chapter 22 8th Edition.pdf, PDF page(s) 12.

List arterial insufficiency characteristics

Tissue ischemia; intermittent claudication progressing to rest pain; decreased or absent pulses; pale skin with elevation and dusky red dependent color; cool temperature; absent or mild edema; thin, shiny, dry, hairless skin; thick ridged nails. Ulcers are generally painful, deep, circular, and well defined on toes, heels, pressure/trauma points, or lateral ankle, with pale ischemic to black/dry gangrenous bases.

Source: Chapter 22 8th Edition.pdf, PDF page(s) 20–21.

List venous insufficiency characteristics

Venous stasis and hypertension; aching/cramping discomfort; generally normal pulses that may be hard to palpate through edema; normal/warm skin; pronounced edema; thick rough skin, brown hemosiderin staining, and stasis dermatitis. Ulcers usually occur around the ankle, especially medially; they are superficial with uneven edges and beefy red to yellow bases and are often painless.

Source: Chapter 22 8th Edition.pdf, PDF page(s) 7–8, 22–23.

Describe a neuropathic ulcer

Develops at pressure points in an area with diminished sensation, commonly with diabetic neuropathy. It may be painless and go unnoticed for some time, increasing infection risk.

Source: Chapter 22 8th Edition.pdf, PDF page(s) 24.

Describe normal and abnormal capillary refill

Normal: Color returns in less than 2 seconds after blanching. Refill exceeding 2 seconds may indicate vasoconstriction, decreased cardiac output, shock, arterial occlusion, or hypothermia.

Source: Chapter 22 8th Edition.pdf, PDF page(s) 13; also Chapter 14 PDF p. 20.

Locate the lower extremity pulses

Femoral: Just below the inguinal ligament, halfway between the pubis and anterior superior iliac spine.

Popliteal: In the popliteal fossa behind the knee.

Posterior tibial: Behind the medial malleolus, between it and the Achilles tendon.

Dorsalis pedis: On the dorsum of the foot, lateral and parallel to the extensor tendon of the great toe; use light pressure.

Source: Chapter 22 8th Edition.pdf, PDF page(s) 17.

Chapter 23 Abdomen

List assessment and quadrant order

Technique order: Inspection → auscultation → percussion → palpation.

Quadrant sequence for auscultation: RLQ → RUQ → LUQ → LLQ, moving clockwise.

Source: Chapter 23 8th edition.pdf, PDF page(s) 12, 14.

What do hypoactive absent and hyperactive bowel sounds indicate

Hypoactive: Fewer than 5 sounds/minute; may occur after surgery, with paralytic ileus, or late bowel obstruction.

Absent: No sounds after adequate listening; suggests absent motility and may occur with paralytic ileus, peritonitis, or obstruction. Requires prompt evaluation.

Hyperactive: More than 30 sounds/minute; may occur with early obstruction, gastroenteritis, or diarrhea. High pitched sounds can occur with obstruction.

Source: Chapter 23 8th edition.pdf, PDF page(s) 14, 16.

What technique is required before declaring bowel sounds absent

Auscultate with the diaphragm through all four quadrants and listen for 5 minutes before determining that bowel sounds are absent.

Source: Chapter 23 8th edition.pdf, PDF page(s) 14.

List preparation for a GI assessment

Have the client empty the bladder. Keep the room warm. Position supine with a pillow under the head. Warm the stethoscope. Assess painful areas last and use distraction to reduce abdominal guarding.

Source: Chapter 23 8th edition.pdf, PDF page(s) 12.

List peptic ulcer disease risk reduction strategies

Practice hand hygiene and cook food completely. Follow medication precautions, including taking with food when directed. Stop smoking or chewing tobacco. Review NSAID exposure and the listed risk factors with the provider; the course also lists alcohol, stress, spicy foods, and H. pylori among risk factors.

Source: Chapter 23 8th edition.pdf, PDF page(s) 24.

List GERD risk reduction strategies

Avoid alcohol; eat small meals; lose excess weight; remain upright for 2 hours after eating. Avoid foods that trigger reflux, including fatty/fried foods, mint, caffeine, spicy foods, tomato dishes, citrus, and chocolate/sweets when symptomatic. Smoking is also a listed risk factor.

Source: Chapter 23 8th edition.pdf, PDF page(s) 25.

What is the main cause of chronic constipation

The chapter identifies several contributors rather than one single main cause: diminished physical activity, inadequate fluid intake, low dietary fiber, and medications such as iron and opioids. Assess these factors and changes from the client's usual bowel routine.

Source: Chapter 23 8th edition.pdf, PDF page(s) 7, 19.

Describe specialty test techniques and indications

Blumberg: Assess rebound tenderness by pressing deeply at 90 degrees between the umbilicus and anterior iliac crest, then releasing. Sharp pain on release suggests peritoneal irritation/appendicitis.

Murphy: Press under the liver border at the right costal margin and ask for deep inspiration. Sharp pain causing interrupted inspiration suggests gallbladder inflammation.

Psoas: With the client lying on the left side, hyperextend the right leg. RLQ pain suggests iliopsoas irritation from appendicitis.

Obturator: Flex the right leg to 90 degrees, stabilize the ankle, and rotate the hip. RLQ pain suggests obturator irritation associated with appendicitis or perforated appendix.

Source: Chapter 23 8th edition.pdf, PDF page(s) 17–18.

Chapter 26 Male Genitourinary

List prostate cancer assessment findings

Trouble urinating, decreased force of urinary stream, blood in urine or semen, pelvic discomfort, bone pain, erectile dysfunction, and unexplained weight loss. Urinary symptoms can overlap with benign prostatic hyperplasia. PSA is not definitive: it may be elevated without cancer or normal in a person with cancer.

Source: Chapter 26 8th edition.pdf, PDF page(s) 14–15.

Describe testicular self examination steps

The course teaches monthly examination. Use both hands, placing the index and middle fingers beneath the testis and the thumb on top. Roll gently in a horizontal plane between the thumb and fingers, then palpate upward along the testis and along the spermatic cord on its posterior aspect. Normal tissue is smooth and uniform. Report any small pea sized lump or other abnormality to the provider.

Source: Chapter 26 8th edition.pdf, PDF page(s) 12.

Differentiate hypospadias and epispadias

Hypospadias: Urethral meatus is displaced to the ventral/underside of the penis.

Epispadias: Urethral meatus is displaced to the dorsal/top surface.

Source: Chapter 26 8th edition.pdf, PDF page(s) 8.

List older adult male considerations

Pubic hair thins, decreases, and grays. Penis and testes decrease in size; testes feel less firm. Scrotum becomes pendulous with fewer rugae and may become excoriated if repeatedly sat on. Sperm production decreases around age 40 but may continue into the 80s–90s. Testosterone production gradually declines after about 55–60.

Source: Chapter 26 8th edition.pdf, PDF page(s) 13.

Chapter 27 Female Genitourinary

List cervical cancer risk factors

HPV infection, smoking, immunosuppression, chlamydia infection, overweight, and family history. The notes emphasize persistent HPV as a major cause and discuss sexual exposure as the route of transmission.

Source: Chapter 27 8th edition.pdf, PDF page(s) 13–15.

List older adult female considerations

Estrogen decreases, causing vaginal thinning, dryness, and reduced natural lubrication. Atrophy increases infection/trauma risk and may make intercourse painful; lubrication can improve comfort. Pubic hair thins, decreases, and grays. Muscle weakness/loss of urethral elasticity may contribute to incontinence. Pelvic floor weakness can lead to uterine prolapse, cystocele, or rectocele. Any postmenopausal vaginal bleeding needs follow up.

Source: Chapter 27 8th edition.pdf, PDF page(s) 20–22.

List minor and serious oral contraceptive side effects

Minor: Weight gain, breast tenderness, headaches, nausea, and increased vaginal yeast infections.

Serious: Stroke, deep vein thrombosis, and myocardial infarction. The notes identify smoking and frequent prolonged travel as additional risk considerations.

Source: Chapter 27 8th edition.pdf, PDF page(s) 7–8.

List hemorrhoid education

Eat a high fiber diet, avoid straining, exercise regularly, and drink plenty of water (the course specifies 6–8 glasses/day). Avoid prolonged sitting, including sitting on the toilet, and respond promptly to the urge to defecate. Avoid anal intercourse. OTC creams/soothing pads and ice packs may provide relief.

Source: Chapter 27 8th edition.pdf, PDF page(s) 19.

Reference Documents

The source lines throughout the worksheet refer to these documents. Page numbers refer to the PDF file page count. Chapter 27 PowerPoint speaker notes corroborate the corresponding PDF notes.

Chapter 14 8th edition.pdf

Chapter 20 8th edition.pdf

Chapter 21 8th Edition.pdf

Chapter 22 8th Edition.pdf

Chapter 23 8th edition.pdf

Chapter 26 8th edition.pdf

Chapter 27 8th edition.pdf

Chapter 27 8th edition.pptx — slides 5, 10, 13–16 and speaker notes.